



WHO
Data



Afghanistan

Health data overview for the Islamic Republic of Afghanistan

Population

41 454 761 (2023)

Current health expenditure (% of GDP)

21.83 (2021)

WHO region

Eastern Mediterranean

World Bank income level

Low income (LIC)

Population, Afghanistan

In Afghanistan, the current **population** is **41,454,761** as of **2023** with a **projected increase of 85%** to **76,885,135** by **2050**.

Population growth rate

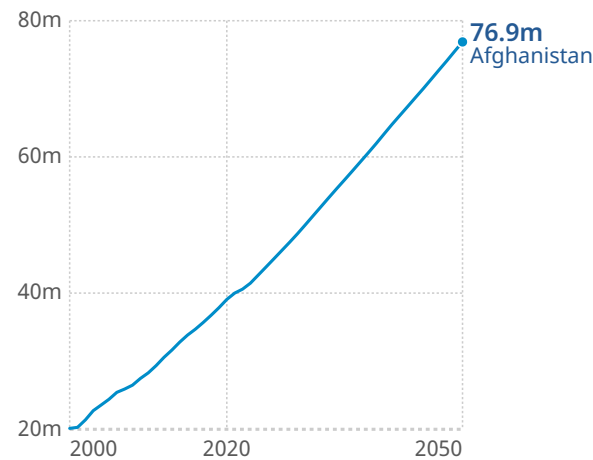
Afghanistan, 2023

2.2%

+0.71 percentage points
change since 2022
rate

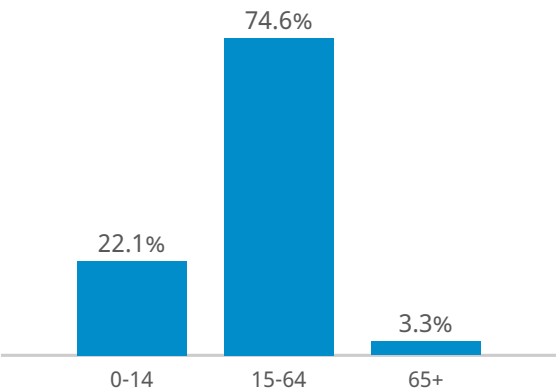
Population trend and projection

Afghanistan, 2000 - 2050



Age distribution of population (%)

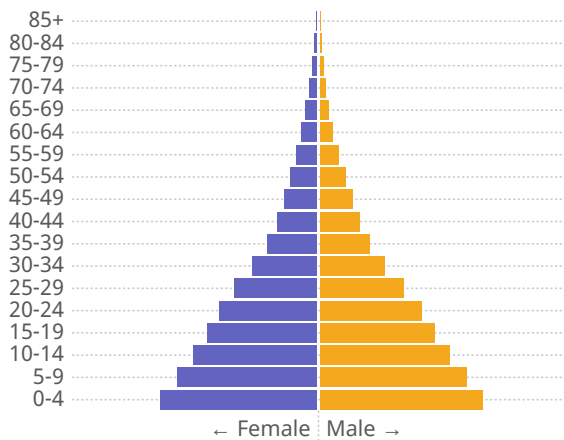
Afghanistan, 2023



Demographic change 2023 - 2050

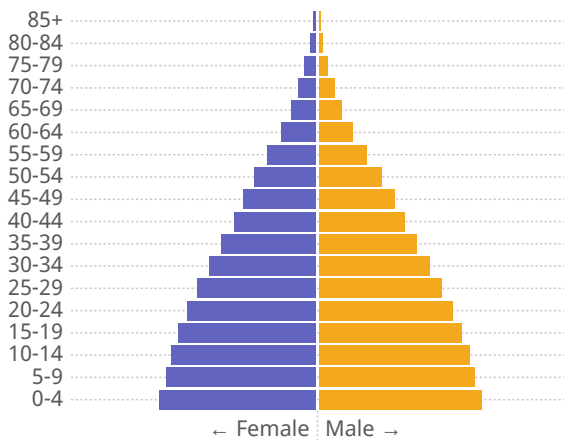
2023

Population by age and sex. Afghanistan



2050

Projected population by age and sex. Afghanistan



Life expectancy, Afghanistan

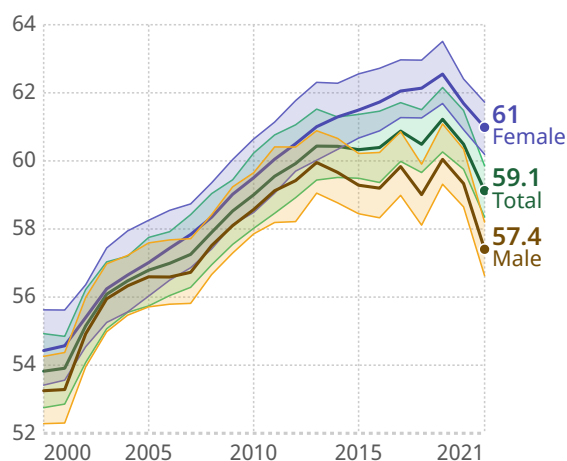


Life expectancy

Healthy life expectancy

Life expectancy at birth

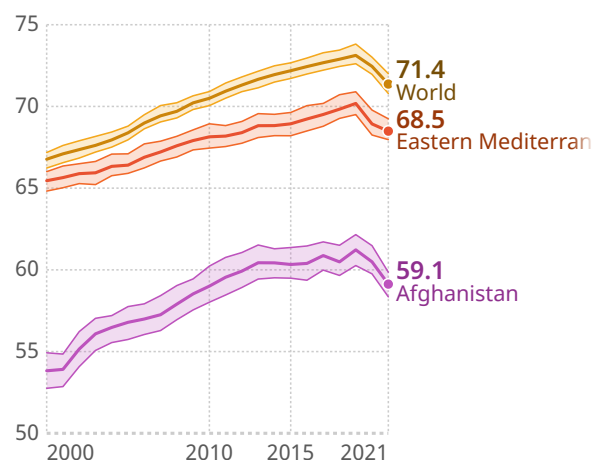
The average number of years that a newborn could expect to live. Afghanistan, by sex, 2000 - 2021.


[Download data](#)

In Afghanistan, **life expectancy at birth (years)** has **improved by ▲ 5.3 years** from 53.8 [52.8 - 54.9] years in 2000 to **59.1** [58.3 - 59.9] years in 2021.

Life expectancy at birth - region / global

The average number of years that a newborn could expect to live. Afghanistan, 2000 - 2021


[Download data](#)

In Eastern Mediterranean, **life expectancy at birth (years)** has **improved by ▲ 3.03 years** from 65.5 [64.8 - 66] years in 2000 to **68.5** [68 - 69.2] years in 2021.

Healthy life expectancy (HALE) at birth

The average number of years that a person can expect to live in “full health” from birth. Afghanistan, by sex, 2000 - 2021.

Total

Healthy life expectancy (at birth), years +4.42

Female

Healthy life expectancy (at birth), years +5.16

Male

Healthy life expectancy (at birth), years +3.7

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In Afghanistan, **healthy life expectancy at birth (years)** has **improved by ▲ 4.42 years** from 46 [44.9 - 47.3] years in 2000 to **50.4** [49.6 - 51.4] years in 2021.

Healthy life expectancy (HALE) at birth - region / global

The average number of years that a person can expect to live in “full health” from birth. Afghanistan, 2000 - 2021

World

Healthy life expectancy (at birth), years +3.79

Eastern Mediterranean

Healthy life expectancy (at birth), years +2.35

Afghanistan

Healthy life expectancy (at birth), years +4.42

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In Eastern Mediterranean, **healthy life expectancy at birth (years)** has **improved by ▲ 2.35 years** from 56.7 [55.7 - 57.5] years in 2000 to **59** [58.2 - 59.9] years in 2021.

What is the difference between life expectancy and healthy life expectancy?

"Life expectancy" refers to the number of years a person can expect to live. It is an important way of assessing the health of a population and is used to inform health policy and initiatives that impact everyday life.

"Life expectancy at birth" is the average number of years that a newborn could expect to live, (the average age of death) while "Healthy life expectancy (HALE) at birth" is the average number of years that a person could expect to live in "full health" from birth. This measurement takes into account years lived in less than full health due to disease and/or injury.

* The lack of complete and reliable mortality data, especially for low income countries and particularly on mortality among adults and the elderly, necessitates the application of modelling to estimate life expectancy. This may lead to minor differences compared with official life tables prepared by Member States.

Leading causes of death, Afghanistan

Top causes of death - Female

Deaths per 100 000 population. Afghanistan, 2021

Ischaemic heart disease	79.4	
COVID-19	68.4	
Maternal conditions	60.3	
Stroke	56.7	
Preterm birth complications	46.8	
Collective violence and legal intervention	44.4	
Lower respiratory infections	35.8	
Measles	34.1	
Tuberculosis	33	
Kidney diseases	24.1	

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Top causes of death - Male

Deaths per 100 000 population. Afghanistan, 2021

Collective violence and legal intervention	197.7	
COVID-19	88.7	
Ischaemic heart disease	81.4	
Preterm birth complications	56.3	
Stroke	42.7	
Measles	38.1	
Road injury	36.8	
Lower respiratory infections	30.5	
Birth asphyxia and birth trauma	27.9	
Tuberculosis	17.3	

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Top causes of death

Deaths per 100 000 population. Afghanistan, 2021

Collective violence and legal intervention	121.8	
Ischaemic heart disease	80.5	
COVID-19	78.8	
Preterm birth complications	51.6	
Stroke	49.6	
Measles	36.1	
Lower respiratory infections	33.1	
Maternal conditions	29.8	
Birth asphyxia and birth trauma	25.5	
Tuberculosis	25.1	

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Share of deaths by broad cause

Afghanistan, 2021

Cause of Death 	% 
Communicable, maternal, perinatal and nutritional conditions	39.1
Noncommunicable diseases	34.2
Injuries	23.2
Other COVID-19 pandemic-related outcomes	3.5

Why and how are causes of death monitored?

Understanding the reasons why people die helps with the understanding of how they lived, in order to improve health services and reduce preventable deaths in every country, responding effectively to changing circumstances. Annual monitoring of causes of death allows countries to address their causes and adapt health systems to react effectively.

The leading causes of death are statistical estimates based on available data. These estimates are produced using data from multiple sources, including national vital registration data, latest estimates from WHO technical programmes, United Nations partners and inter-agency groups, as well as the Global Burden of Disease and other scientific studies. Before publishing, they are reviewed by WHO Member States through consultation with national focal points and WHO country and regional offices.

The top ten causes of death are statistical estimates based on available data. Death registration data is unavailable or unusable for Afghanistan due to quality issues. Estimates of mortality by cause should be interpreted with caution. These estimates may be used for priority setting, however, they are not likely to be informative for policy evaluation or comparisons among countries.

Health statistics, Afghanistan



Health status Risk factors Service coverage Health systems

New HIV infections

Afghanistan, 2023

Data not available

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indicator



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Tuberculosis incidence

Afghanistan, 2023

204 [117 - 400]

-2
improving since 2022

Tuberculosis incidence (per 100 000 population)

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Malaria incidence

Afghanistan, 2022

9.19 [7.09 - 11.68]

+2.93
worsening since 2021

Malaria incidence (per 1000 population at risk)

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indicator



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data



Hepatitis B

Afghanistan, 2020

Data not available

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indicator



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data



Probability of dying from non-communicable diseases (NCDs)

Afghanistan, 2019

34.4% [20.2% - 50.3%]

–0.2 percentage points
improving since 2018

Probability of dying from any of cardiovascular diseases, cancer, diabetes, chronic respiratory diseases between age 30 and exact age 70 (%)

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indicator



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Road traffic deaths

Afghanistan, 2021

24.1

Road traffic mortality rate (per 100 000 population)

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indicator



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Unintentional poisoning

Afghanistan, 2019

2.89 [1.17 - 5.61]

+0.16
worsening since 2018

Mortality rate from unintentional poisoning (per 100 000 population)

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indicator



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data



Suicide deaths

Afghanistan, 2019

3.53 [1.97 - 6.44]

–0.03
improving since 2018

Suicide mortality rate (per 100 000 population)

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indicator



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Homicide

Afghanistan, 2019

8.48

[4.7 - 14.37]

-0.18

improving since 2018

Mortality rate due to homicide (per 100 000 population)

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Maternal Mortality

Afghanistan, 2023

520.5

[338.6 - 942]

-80.6

improving since 2019

Maternal mortality ratio (per 100 000 live births)

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Under five mortality

Afghanistan, 2022

57.39

[44.82 - 73.12]

-1.94

improving since 2021

Under-five mortality rate (per 1000 live births)

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Neonatal mortality

Afghanistan, 2022

35.17

[27.3 - 45.09]

-0.84

improving since 2021

Neonatal mortality rate (per 1000 live births)

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Mortality due to air pollution

Afghanistan, 2019

269.1 [222.9 - 316.1]

-52.7
improving since 2010

Age-standardized mortality rate attributed to household and ambient air pollution (per 100 000 population)

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Mortality due to unsafe WASH

Afghanistan, 2019

16.6

Mortality rate attributed to exposure to unsafe WASH services (per 100 000 population)

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Hypertension

Afghanistan, 2019

40.2% [33.7% - 46.9%]

+0.3 percentage points
worsening since 2018

Age-standardized prevalence of hypertension among adults aged 30-79 years (%)

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Tobacco use

Afghanistan, 20222

22.9% [17% - 28.8%]

Age-standardized prevalence of tobacco use among persons 15 years and older (%)

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Alcohol consumption

Afghanistan, 2020

0.01

[0 - 0.1]

-0

change since 2019

Total alcohol per capita (≥ 15 years of age) consumption (litres of pure alcohol)

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Adult obesity

Afghanistan, 2022

19.2%

[16.3% - 22.2%]

+1 percentage points

worsening since 2021

Age-standardized prevalence of obesity among adults (18+ years) (%)

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Children and adolescents obesity

Afghanistan, 2022

8.5%

[4.4% - 14.1%]

+0.6 percentage points

worsening since 2021

Prevalence of obesity among children and adolescents (5–19 years) (%)

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Intimate partner violence (previous 12 months)

Afghanistan, 2018

Data not available

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Download data

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Intimate partner violence (Lifetime)

Afghanistan, 2018

Data not available

Visit indicator

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Download data

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Stunting under 5

Afghanistan, 2022

Data not available

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Wasting under 5

Afghanistan, latest

3.6%

[3.3% - 3.9%]

-13.7 percentage points

improving since 1997

Prevalence of wasting in children under 5 (%)

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Safely managed drinking water

Afghanistan, 2022

29%

+1 percentage points

improving since 2021

Proportion of population using safely-managed drinking-water services (%)

Visit indicator

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Safely managed sanitation

Afghanistan, 2022

24%

+1 percentage points

improving since 2021

Proportion of population using safely-managed sanitation services (%)

Visit indicator

→

Download data

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Handwashing facilities

Afghanistan, 2022

50%

+1 percentage points

improving since 2021

Proportion of population using a hand-washing facility with soap and water (%)

Visit indicator

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Download data

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Safely treated wastewater flows
Afghanistan, 2022

Data not available

[Visit indicator](#) →

[Download data](#) 

Clean fuels
Afghanistan, 2022

31%
[17% - 47%]
±0 percentage points
stable since 2021

Proportion of population with primary reliance on clean fuels and technology (%)

[Visit indicator](#) →

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Fine particulate matter
Afghanistan, 2019

60.1
[44.1 - 83.5]
-11
improving since 2018

Annual mean concentrations of fine particulate matter (PM2.5) in urban areas (µg/m3)

[Visit indicator](#) →

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Interventions against NTDs
Afghanistan, 2022

17 million
+2.6 million
change since 2022
Reported number of people requiring interventions against NTDs

Visit indicator

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Family planning
Afghanistan, latest

42.1%
Proportion of women of reproductive age who have their need for family planning satisfied with modern methods (%)

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Births attended by skilled health personnel
Afghanistan, latest

68%
+54 percentage points
improving since 2003
Proportion of births attended by skilled health personnel (%)

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⬇️

DTP3 immunization coverage among 1-year-olds
Afghanistan, 2023

60%
+2 percentage points
improving since 2022
Diphtheria-tetanus-pertussis (DTP3) immunization coverage among 1-year-olds (%)

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https://data.who.int/countries/004

17/25

Domestic general government health expenditure

Afghanistan, 2021

4%
-0.3 percentage points
worsening since 2020

Domestic general government health expenditure (GGHE-D) as percentage of general government expenditure (GGE) (%)

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Development assistance to medical research and basic health sectors

Afghanistan, 2022

4.81
Total net official development assistance to medical research and basic health sectors per capita (US\$), by recipient country

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Density of doctors

Afghanistan, 2022

2.53
+0.07
improving since 2012
Density of medical doctors (per 10 000 population)

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Density of nurses

Afghanistan, 2022

4.51
+1.96
improving since 2013
Density of nursing and midwifery personnel (per 10 000 population)

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Density of dentists

Afghanistan, 2022

0.06

+0.03

improving since 2014

Density of dentists (per 10 000 population)

Visit indicator

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Density of pharmacists

Afghanistan, 2022

0.37

+0.09

improving since 2012

Density of pharmacists (per 10 000 population)

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Health target progress, Afghanistan

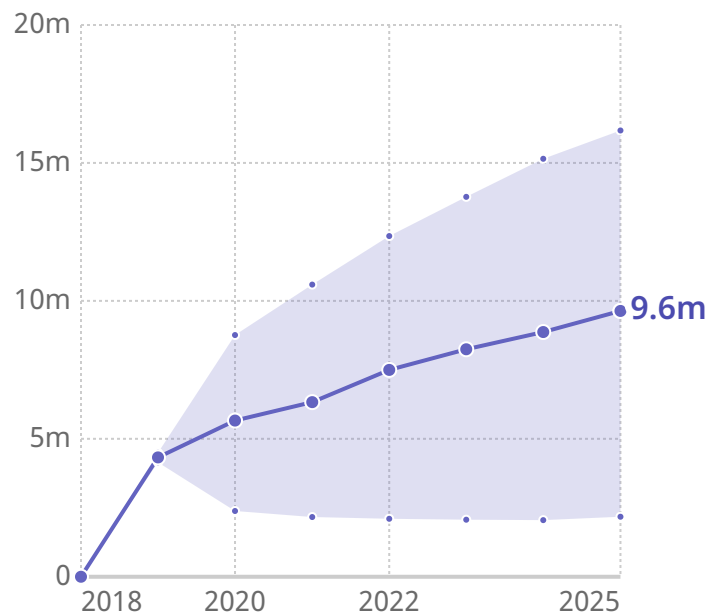
WHO Triple billion targets

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Healthier Populations

In Afghanistan, the number of additional people expected to be enjoying better health and wellbeing is projected to be 9.6m (2.2m – 16.2m) by 2025 compared to 2018.

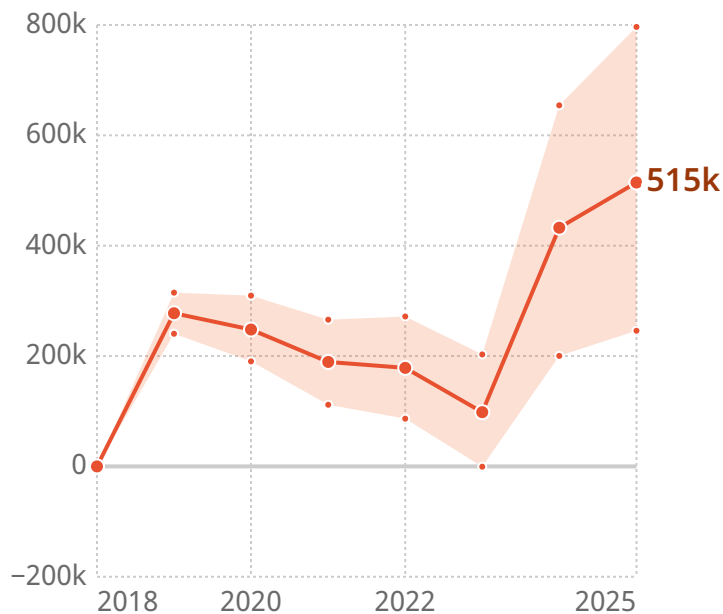
Afghanistan, 2018 - 2025



Universal Health Coverage

In Afghanistan, the number of additional people expected to be covered by essential services and not experiencing financial hardship is projected to be 514.5k (245.9k – 796.5k) by 2025 compared to 2018.

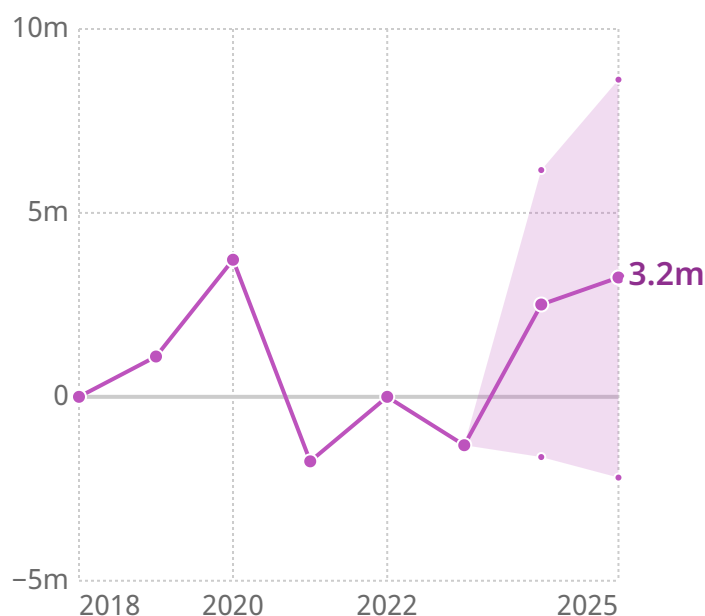
Afghanistan, 2018 - 2025



Health Emergencies Protection

In Afghanistan, the number of additional people expected to be protected from health emergencies is projected to be 3.2m (-2.2m - 8.6m) by 2025 compared to 2018.

Afghanistan, 2018 - 2025



What are the triple billion targets?

The WHO Triple Billion targets measure impact on people's health to ensure that more people are enjoying better health and wellbeing, are benefitting from Universal Health Coverage (UHC) and are better protected from health emergencies.

The estimates are underpinned by 46 outcome indicators. They include 39 Sustainable Development Goal (SDG) indicators and seven non-SDG indicators. These offer a flexible approach to each of the Triple Billion targets, allowing countries to prioritize certain indicators based on their national health strategy, ensuring the Triple Billion strategy remains relevant for all Member States and their unique health challenges.

Further resources

National Statistics Office 

WHO country website 

WHO Eastern Mediterranean 

WHO Global country page 

WHO Global 

Reference metadata

Reference data

ISO alpha 2 code

AF

ISO alpha 3 code

AFG

M49 code

004

WHO short name

Afghanistan

Formal name

the Islamic Republic of Afghanistan

Administrative level

Admin 0

Regional groupings and country classifications

WHO region

Eastern Mediterranean

UN region

Asia > Southern Asia

World Bank income level

Low income (LIC)

Data sources

Population data

[UNDESA population division, World Population Prospects 2022](#), Extracted: 15 May 2024

Current Health Expenditure (CHE) as % of GDP

[World Health Organization, Global Health Expenditure database](#), Extracted: 8 January 2024

Life expectancy and Healthy life expectancy (HALE)

[World Health Organization, Global Health Estimates](#), Extracted: 8 January 2024

Causes of death

[World Health Organization, Global Health Estimates](#), Extracted: 22 November 2022

Triple Billion

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